جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ناديه محمد عبداللطيف بيومى
المحافظة : الجيزه **الحي / المدينة :** اخري **العنوان :** مممممممممممممم
رقم البطاقة : 26806052100588 **رقم التليفون :** 01008592061 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 05-06-1968 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ناديه محمد عبداللطيف بيومى
National ID: 26806052100588

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
ناديه
محمد عبداللطيف بيومى
قرية المتصورية
منشاه القناطر - الجيزه
٢٦٨٠٦٠٥٢١٠٠٥٨٨
JE1755843


٢٩٨٠٦٠٥٢١٠٠٥٨٨
٢٠٢١/١٢
بدون عمل
انثى مسلمة متزوجة
فرج عبدالكريم عطيه خميس
البطاقة سارية حتى ٢٠٢٨/١٢/١٧
١

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : ناديه محمد عبداللطيف محمد
2-ID number: 26806052100588

Sex:

3- Medical number: 83048283
Female
Patient Age : 54 Y

Anat m. palpable

MDT

Family history		Phone	01013998702
BMI		NRS Score	
GROUP		marital status	
Menopausal status	POST	Co morbidity	
Date of MDT	02-Nov-2022	REASON OF MDT	REMDT

Clinical data

Breast Imaging :
12/2021
RT BREAST SPECULATED PATHOLOGICALLY PROVEN MALIGNANT MASS LESION OF SIZE , SIGNAL PATTERN AND EXTENSION AS DETAILED BIRDS6 COCURRENT MULTIPLE ALNS

Mets Workup :

BX:	IHC:	ER:	PR:	Her 2 Neu :	Ki67 :
IDC GII 2/2022		0/8	0/8	+VE	50

T	N	M
4	1	1

Sonomammography
SM 8/2022

RT BREAST UIQ PARTIAL RADIOLOGICL REPONSE BIRADS 6

Recommendations:
A CASE OF MET RT BREAST CA WITH PULMONARY NODULES
RECEIVED 20 WK TAXOL+ 8 CYCLES HERCEPTIN STARTED LAST ON 9/8/2022
CTS 7/22-->RD OF RT BREAST MASS 1.9X4.3 VS 3X4.5 CM
RT AXILLARY LN 1X1
SD OF BILATERAL PULMONARY NODULES AND BULKY LT ADRENAL GLAND.
MDT 8/2022--> FOR 6 WKS TAXOL+CARBO+HERCEPTIN LAST ONE WAS ON 11/10/2022

MDT OUTCOME & PLAN

For new metastatic workup & reevaluation by MDT.

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	

Surgery Consult

Name : Nadia Mohammed Abdel Latif

Date : 9-4-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE BRAIN, CHEST, ABDOMEN & PELVIS REVEALED: -

** *Follow up status , as compared to last study dated 16-2-2022*

1. Brain:

- No focal cerebral areas of abnormal density or abnormal enhancement pattern.
- Preserved normal gray-white matter interface.
- No extra-axial or intra-cerebral areas of fresh blood densities.
- Normal extra-axial CSF spaces & basal skull cisterns.
- Normal size and configuration of the ventricular system.
- No midline shift.
- Unremarkable posterior fossa structures.

2. Chest:

- **Regressive course of the fore-noted:**
  - ✓ Right breast irregular speculated mass lesion inseparable from the underlying right pectoral muscle, measuring 3 x 4.5 cm compared to 11.5 x 8.5 cm.
  - ✓ Right breast skin thickening.
  - ✓ Right axillary level I and II pathologically enlarged lymph nodes, the largest measures 1 x 1 cm compared to 3.3 x 2.7 cm.
- **Unchanged** few bilateral scattered pulmonary nodules, the largest is noted at lateral segment of right lower lung lobe measuring 7 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

3. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.

Name : Nadia Mohammed Abdel Latif

Date : 16-2-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

** *Follow up status , as compared to last study dated 22-12-2021*

1. Chest:

- **Progressive course of the fore-noted:**
  - ✓ Right breast irregular speculated mass lesion inseparable from the underlying right pectoral muscle, measuring 11.5 x 8.5 cm compared to 6.8 x 5.2 cm. right breast skin thickening.
  - ✓ Right axillary level I and II pathologically enlarged lymph nodes, the largest measures 3.3 x 2.7 cm compared to 2.7 X 2.4 cm.
- **Again noted** enlarged right internal mammary lymph node measuring 10 mm.
- **Unchanged** few bilateral scattered pulmonary nodules, the largest is noted at lateral segment of right lower lung lobe measuring 7 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal gland.
- **Stationary** bulky left adrenal with internal calcifications.
- Under-filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic lymphadenopathy or collections.
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

OPINION:

Follow up status showing:

- ***Progressive course of the right breast locally advanced malignant looking mass lesion and the metastatic ipsilateral axillary lymph nodes as described.***
- ***Stationary rest of the findings since last dated CT study.***

Dr. M. Gomaa, M.D.

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

atient Name	: الأستاذه / نادية محمد عبداللطيف بيومى	Request No.	: 139136
atient No.	: 26806052100588	Request Date	: 16/01/2022
ıth No.	: 169/2022	Verification Date	: 02/02/2022
ex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
ge	: 53 Y	Department	: Out Patient
eferreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass

Nature of the specimen: Tru-cut biopsy

Gross Examination: Multiple greyish white and hemorrhagic tissue cores up to 1 cm, totally submitted.
Procedure: Tru-cut biopsy

Microscopic examination:
Histological type: Invasive duct carcinoma
Grade (differentiation+ nuclear pleomorphism + mitosis): GII (2+2+2)
Angioinvasion: N/A
Ductal carcinoma in situ:N/A

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma GII
For IPT (ER, PR, Her2, Ki-67)

Prof. Aml Hareedy
Dr. Mennatullah Atef

Dr. Walaa Ghonaim

Verified by :
walaa ghoniem

Room No. (4) - First Floor, Building No. 987, 9th Corniche - Nile Street - Mit El Qudessia - Cairo, Egypt

Patient Name	: الأستاذه / نادية محمد عبداللطيف بيومى	Request No.	: 139136
Patient No.	: 26806052100588	Request Date	: 16/01/2022
Path No.	: 169/2022	Verification Date	: 06/02/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 53 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass

Nature of the specimen: Tru-cut biopsy

Gross Examination: Multiple greyish white and hemorrhagic tissue cores up to 1 cm, totally submitted.
Procedure: Tru-cut biopsy

Microscopic examination:
Histological type: Invasive duct carcinoma
Grade (differentiation+ nuclear pleomorphism + mitosis): GII (2+2+2)
Angioinvasion: N/A
Ductal carcinoma in situ:N/A

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma GII
- IPT:

ER: No nuclear reaction in tumour cells. Negative (Allred score 0/8)
PR: No nuclear reaction in tumour cells. Negative (Allred score 0/8)
Her-2: Strong complete membrane reaction in tumour cells. Positive, score 3+
Ki67: 50%

Prof. Dr. Amal Hareedy
Dr. Mennatullah Atef
Dr. Walaa Ghonaim
Dr. Abdulrahman Ibrahim

Verified by :

شارع رقم (4) - الدور الأول - مبنى رقم (937) - قرطاس الثين - مدينة القبة - القاهرة - مصر
House No. (4) - First Floor-Building No. 937- Nile Corniche - Sitte El Qadeem- Cairo, Egypt

Kasr El-Ainy Center of Radiation Oncology and Nuclear Medicine

Faculty Of Medicine, Cairo University

"NEMROCK"

مركز قصر العيني

لعلاج الأورام والطب النووي

كلية الطب - جامعة القاهرة

(نمروك)

Nuclear Medicine Unite

Report

Patient Name:	نادية mohammad Abd El Latif	Patient Gender:	Female
Patient Code:	Q34940	DATE:	1/5/2023 11:10:38 AM

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

Clinical history

- A 52-year-old patient presented with history of right breast cancer, for assessment.

Technique

- *Anterior and posterior whole body scans were acquired 2 hours post IV. injection of 740 MBq of the tracer.*

Findings

- No abnormal osseous tracer uptake detected that would account for serious osseous lesions.
- Degenerative changes of the spine are noted.
- Linear warm tracer is seen in mid shaft of left tibia posteriorly? Traumatic **for clinical correlation**.
- Osteoarthritic changes of the both knees are seen.

Opinion

- **No scintigraphic evidence of serious bone lesions.**

Dr. *Shrouk Atef* *Dr.Maha Salama, MD*

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Mohamed Abdel Latteef Bayoumi	Date : 03/11/2022
Exam : C.T. ABDOMEN &PELIVS WITH CONRAST	Age : 54 Year(s)
Medical ID:	Pat. ID: 15924

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

***** Follow up status, as compared to previous study dated 31/07/2022***

1. ***Chest:***

- **Regressive course of the fore-noted:**
  - Right breast irregular speculated mass lesion inseparable from the underlying right pectoral muscle, measuring 1.4 x 3.3 cm compared to 1.9 x 4.3 cm.
- Mild regressive course regarding the previously seen right axillary level I prominent lymph nodes, the largest still measures 1 x 0.9 cm compared to 11 x 12 mm.
- **Unchanged** few bilateral scattered pulmonary nodules, the largest is noted at lateral segment of right lower lung lobe measuring 7.5 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is enlarged with homogenous low CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal gland.
- **Stationary** bulky left adrenal with internal calcifications.

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Mohamed Abdel Latteef Bayoumi	Date : 25/10/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 54 Year(s)
Medical ID: 5481/2021	Pat. ID: 15924

FOLLOW UP RIGHT BREAST ULTRASOUND

INDICATION:

- Follow up post chemotherapy for right breast neoplastic process.
- In correlation to the previous study of 25-08-2022.

REPORT:

- right breast UIQ at 1 o'clock heterogenous irregular mass, measured 1x2cm, (was 2x1cm in prior study dated 08/2022
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Multiple enlarged axillary level I LNs with effaced hilum and cortical thickening reaching about 5mm

OPINION:

- Follow up post chemotherapy for a case of right breast malignancy, showing stationary course regarding the mass and progressive regarding the LNS (BIRADS 6)

- Signature Dr. Walaa Abdelazez
- 25/10/2022 12:58 PM

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Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Mohamed Abdel Latteef Bayoumi	Date : 25/08/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 54 Year(s)
Medical ID: 169/2022	Pat. ID: 15924

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right breast pathologically proven malignant mass, on NACT

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast UIQ deeply seated posterior third asymmetry, along with right breast global asymmetry with round calcifications, with no available prior studies for comparison
- Complementary ultrasound showed right breast UIQ at 1 o'clock heterogenous irregular mass, measured 1x2cm, (was 2x2.7cm in prior study dated 12/2021
- no left breast solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LNs.
- No right axillary pathological LNs, was 3 pathological in prior study
- Free right infra and supra-clavicular regions

Opinion:

- Right breast UIQ partial radiological response (BIRADS 6)
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

25/08/2022 12:47 PM

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Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Mohamed Abd Al Latif Bayomy	Date : 31/07/2022
Exam : C.T CHEST WITH CONTRAST	Age : 54 Year(s)
Medical ID:	Pat. ID: 15924

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

** *Follow up status, as compared to previous study dated 9-4-2022*

1. **Chest:**
   - **Regressive course of the fore-noted:**
     - ✓ Right breast irregular speculated mass lesion inseparable from the underlying right pectoral muscle, measuring 1.9 x 4.3 cm compared to 3 x 4.5 cm.
     - ✓ Right breast skin thickening.
   - Right axillary level I prominent lymph nodes, the largest still measures 1 x 1 cm.
   - **Unchanged** few bilateral scattered pulmonary nodules, the largest is noted at lateral segment of right lower lung lobe measuring 7 mm.
   - No pathological hilar or mediastinal lymph nodes.
   - No evidence of pleural effusion.
   - Average cardiac size. No pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
2. **Abdomen and pelvis:**
   - The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
   - Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - No pathological abdominal or pelvic lymph nodes.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
   - Normal appearance of the pancreas, aorta, I.V.C and right supra-renal gland.
   - **Stationary** bulky left adrenal with internal calcifications.
   - Under-filling of the urinary bladder with no filling defects or diverticular outpouching.

Name : Nadia Mohammed **Date :** 22-12-2021

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. **Chest:**

- Right breast irregular speculated mass lesion inseparable from the underlying right pectoral muscle, measuring 3.8 X 3.5 cm. right breast skin thickening.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 2.7 X 2.4 cm.
- Enlarged right internal mammary lymph node measuring 9 mm.
- Few bilateral scattered pulmonary nodules, the largest is noted at lateral segment of right lower lung lobe measuring 7 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. **Abdomen and pelvis:**

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

OPINION:

- ***Right breast locally advanced malignant looking mass lesion with metastatic ipsilateral axillary lymph nodes as described.***
- ***Bilateral scattered pulmonary nodules worrisome for metastatic disease.***
- ***No definite hepatic or osseous metastatic deposits.***

Dr. Mohammed Mohsen, MSc.

Dr. Amr Farouk, M.D.

Pat. Name: Nadia Mohamed Abd Al Latif	Pat. ID: 388736
Pat. Age 52 Y	Pat. Sex F
Ref. Phy. Dear Prof	Study Date 20/12/2021

DYNAMIC MRI OF THE BREASTS REVEALS :-

Technique: -

- *Variable pulse sequences*
- *Fat suppression T2w (STAIR / SPAIR)*
- *Dynamic post contrast study*
- *Time intensity curve interpretation*

Findings :-

Known case of right breast mass lesion , pathologically proven ductal adenocarcinoma , no previous examination for comparison , however the current study revealed

- Both breast are of involuting type with sparse glandular tissue in abundance of fatty components

- Rather circumscribed right breast retro-glandular irregular speculated solid mass lesion is seen measuring about 50 x 60 x 67 mm . It exhibits inconspicuous iso- hypointense pattern T1w , inhomogenous predominant hyperintense signals on T2w and relative bright signals on STAIR sequences . Appreciable heterogenous enhancement on post contrast study . Restricted (bright) signal on DWI and hypointense signals on ADC map . Time intensity curve interpretation revealed type II curve pattern with rapid uptake / plateau pattern . This lesion is seen invading the underlying chest wall musculature fuzzy indistinct surrounding parenchyma with Thickened adjacent glandular tissue as well as skin and subcutaneous tissue with retracted deviated nipple

- Multiple variable sized globular axillary lymph nodes the largest measuring about 30 x 17
- Preserved integrity of the left breast with no suspicious cystic or solid lesions

OPINION

-Right breast speculated pathologically proven malignant mass lesion of size , signal pattern and extension as detailed (BIRAD-6) . Concurrent multiple globular axillary Lns likely infiltrated

REGARDS ,,

Dr. Ossama Mohammady , MD

Ministry Of Health and Population
Sheikh Zayed specialized Hospital
Radiology Department
Ultrasound & Color Doppler unit

وزارة الصحة والسكان
مستشفى الشيخ زايد التخصصى
قسم الأشعـــة
وحدة الموجات فوق الصوتية والدوبلر الملون

Patient name: ***Mrs. Nadia Mohammed Abdel Latif***
Date: ***2021-12-15***

BILATERAL BREAST SONOMAMMOGRAPHY

Findings:

- ***A known case of pathologically proven right breast ductal carcinoma, sonomammography revealed:***
- ***An UIQ right beast hard non-mobile hypoechoic iregular ill defined speculated solid mass lesion, measuring roughly about 3x2.5cm, surrounded by marked edematous changes.***
- ***The mass shows indistinct deeper extension for further MRI assessment.***
- ***About 3 right axillary rounded pathological lymph nodes are seen with eccenteric hilum, largest about 2.7x2.2cm.***
- *Unremarkable rest of right breast parenchyma.*
- *Free left side.*

Opinion:

- ***Right breast UIQ mass lesion as described with pathological right axillary lymphadenopathy (BI-RADS 6)***

Much obliged,
Dr. Ahmed Kamal_FEBR.
Dr.Hamsa El Shahat
Time of scan 11:20 AM.

BI-RADS Assessment Categories are:

- 0: Incomplete
- 1: Negative
- 2: Benign finding(s)
- 3: Probably benign
- 4: Suspicious abnormality
- 5: Highly suggestive of malignancy
- 6: Known biopsy – proven malignancy.

Pat. Name: Nadia Mohamed Abd Al Latif	Pat. ID: 388736
Pat. Age 52 Y	Pat. Sex F
Ref. Phy. Dear Prof	Study Date 20/12/2021

DYNAMIC MRI OF THE BREASTS REVEALS :-

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Known case of right breast mass lesion , pathologically proven ductal adenocarcinoma , no previous examination for comparison , however the current study revealed

- Both breast are of invoiuting type with sparse glandular tissue in abundance of fatty components

- Rather circumscribed right breast retro-glandular irregular speculated solid mass lesion is seen measuring about 50 x 60 x 67 mm . It exhibits inconspicuous iso- hypointense pattern T1w , inhomogenous predominant hyperintense signals on T2w and relative bright signals on STAIR sequences . Appreciable heterogenous enhancement on post contrast study . Restricted (bright) signal on DWI and hypointense signals on ADC map . Time intensity curve interpretation revealed type II curve pattern with rapid uptake / plateau pattern . This lesion is seen invading the underlying chest wall musculature fuzzy indistinct surrounding parenchyma with Thickened adjacent glandular tissue as well as skin and subcutaneous tissue with retracted deviated nipple

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Mohamed Abdel Latteef Bayoumi	Date : 26/01/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 56 Year(s)
Medical ID: 5481/2021	Pat. ID: 15924

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had Right MRM in

Technique

- left breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free Right operative bed with no evidence of recurrent mass lesions
- Free Right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. basma mohmaed

26/01/2025 3:20 PM

Ultrasound
[[DOCTYPE ULTRASOUND]]

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Patient Name: Nadia Mohamed Abdel Latteef Bayoumi	Date : 19/01/2025
Exam : C.T ABDOMEN &PELVIS	Age : 56 Year(s)
Medical ID: 5481/2021	Pat. ID: 15924

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

** *Follow up status, as compared to previous pre-operative study dated 03/11/2022*

1. Chest:

- Status post right mastectomy showing no sizable operative bed masses.
- **Unchanged** few bilateral scattered pulmonary nodules, the largest is noted at lateral segment of right lower lung lobe measuring 7.5 mm becoming more solid in current study.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous low CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal gland.
- **Stationary** bulky left adrenal with internal calcifications.
- Under-filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic lymphadenopathy or collections.
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

OPINION:

Follow up study the current non contrast study is showing:

CT Scan
[[DOCTYPE.CT_SCAN]]

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Patient Name: Nadia Mohamed Abdel Latteef Bayoumi	Date : 19/01/2025
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- ***Stable size of the bilateral pulmonary nodules with more solid appearance of the right basal lateral segment nodule for close follow up.***
- ***Stationary rest of the findings.***

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE.CT_SCAN]]

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مستشفى اورام دار السلام
اسم المريض رباعي :
رقم الملف الطبي :
نوع الوجبة :
تاريخ اعداد الوجبة :
وقت اعداد الوجبة :

eport

Name:
Date: / / 2021

EF	68	55-75%	PWT	<8	0.6-1.1cm
LVEDD	5,0	3.5-5.8cm	AO.R	2,3	2-3.7cm
LVESD	3,1	2-4cm	LA	3,0	2-4cm
FS	~~35~~	25-45%	RV	/	0.7-2.3cm(supine)
SWT	<8	0.6-1.1cm	AO.CUSP	/	1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function Normal

SWMA Nn

LA and Aortic root dimensions Normal

Mitral Valve normal

Aortic valve normal

Tricuspid valve normal

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

D/ Msc cardiology

Echo
[[DOCTYPE ECHO]]